

Axle *Health*

A home-health-native scheduling engine with the most complete workflow in the field: who, what day and what time, recovery from call-outs, zip-level capacity and patient texting are all live. Its one blocker is data. Axle switched off its HCHB integration because HCHB data arrives too late, and at Compassus it arrives every three hours.

SCORECARD GOING IN 63 Conditional · hold · no open stop-checks	LIVE FOOTPRINT ~25 orgs 80% home health. Largest 11k ADC (from their return)	GUIDE QUESTIONS 6 / 18 answered in full; 8 partly; 4 not asked	NEXT TOUCHPOINT Oct 19 Week of the on-site summit. Pilot Q1 2027, full rollout by end of 2027
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Six things that matter

- 01 **It speaks home health natively.** Fulfillment windows on every visit, the Medicare week, front-loading, visit spacing within an order, SOC-before-eval checks, points by service code and end-of-episode queues were all shown in software, not described. 0:52:06
- 02 **The integration blocker is our data, not their code.** Axle needs new-patient data within ~10 minutes and won't go live on a feed it can't predict. HCHB's fastest refresh is 15 minutes and flaky; Compassus runs every 3 hours. 0:19:11
- 03 **Our intake platform changes the math.** Evan offered a real-time feed of patients and ordered disciplines from the Commure-built intake platform. Axle called it "great news." Routine visit plotting still runs through HCHB, so the 3-hour cadence still bites there. 0:18:25
- 04 **Call-out recovery was run live.** One clinician out sick, five visits, and a recommendation for each one: keep the date and change the clinician, or keep the clinician and change the date. Every factor behind it is visible, and it is submitted in seconds. 1:43:00
- 05 **They won't remove the human.** Designed to decide, then rebuilt to advise. Every customer keeps a person in the loop. Adam argued that fully automating the clinician relationship loses more capacity than it saves in headcount. 1:14:22
- 06 **Patient engagement: confirm, don't ask.** Two-way SMS, no patient app, replies route to the clinician. They deliberately send confirmations rather than questions, and collect only when patients are *not* available. Efficient, but it keeps the negotiation with the clinician. 0:41:38

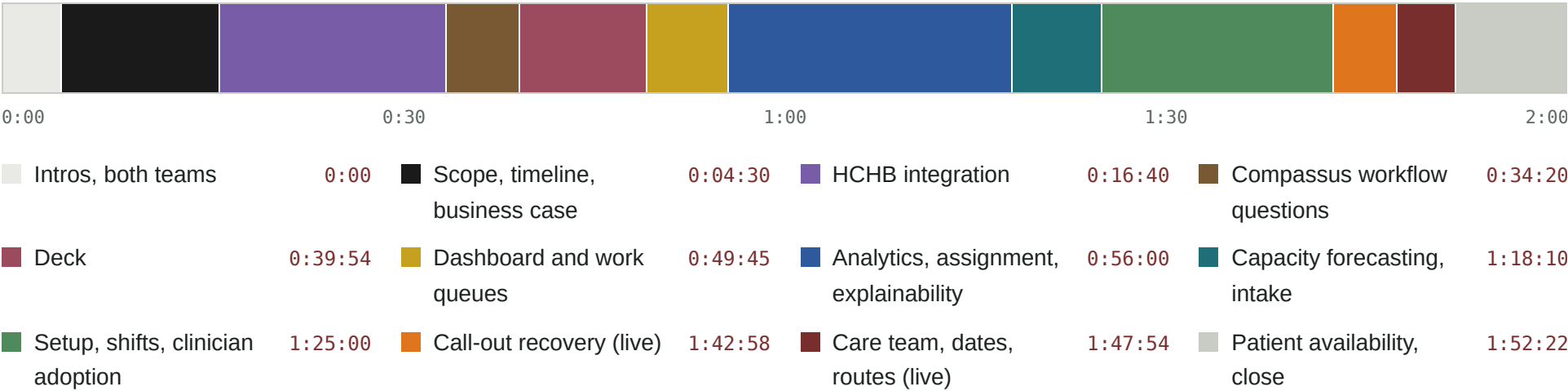
SUGGESTED SCORECARD MOVEMENT

Lift the hold and invite to the on-site, with the data plan as the condition.

The product answered the fluency test that VitalisCare failed, live and in our vocabulary. The Conditional band was set on a zero for integration, and the demo showed that zero is a data-cadence problem Compassus already owns and is working on. Advance on a joint plan: a real-time intake feed for new patients, a target HCHB refresh for orders and frequencies, and a date to turn write-back on.

Partnership terms, company size and funding were not covered on the call. Their return offers referral credits and permanently discounted co-built features. Treat that as the opening position.

The shape of the call



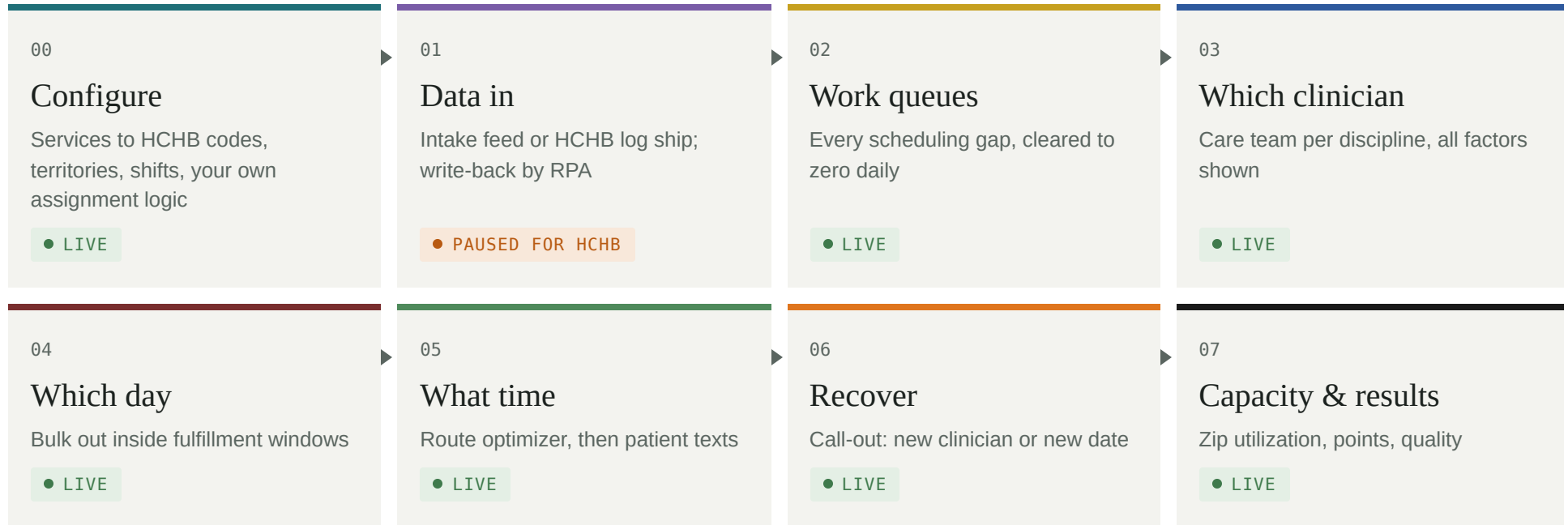
Axle asked the questions for the first 40 minutes: our census, business case, HCHB cadence and who plots routine visits. Then about an hour of live product, much of it spent on a long discussion of explainability and clinician autonomy.

Our numbers, as stated in the room

ITEM	AS STATED	WHO / WHEN
Home health census	24,000 ADC · 85 branches · 19–20 states · 4–5k clinicians (to confirm)	0:03:55
Schedulers	~330–340, home health and hospice combined, local to each branch. Headcount reduction and a central model are goals	0:14:22 · 0:34:31
Clinician pay	~80% paid per visit; some salaried, some hourly	0:19:17
Mileage	45 million miles a year at \$0.60 a mile; a 10% cut is material	0:12:47
HCHB refresh	Every 3 hours (moved from 6 for another AI project). Data architecture work underway	0:18:25 · 0:30:44
Routine plotting	Clinician writes the frequency, a scheduler workflow in HCHB plots it, and the clinician phones patients for times	0:24:29
Timeline	On-site week of Oct 19 → pilot Q1 2027 for 60–90 days → rollout Q2 → complete by end of 2027	0:06:02
Not yet known	Missed-visit rate; exact clinician count; any branch incentives tied to LUPA or compliance (ad hoc today)	0:14:22 · 0:37:44

Who, what day, what time, then recover

Axle splits scheduling into three decisions, *which clinician*, *which day* and *what time*, and lets each organization choose who owns each one. Their default: the office picks the clinician, the office or the clinician picks the day, and the clinician picks the time. Capacity sits above the whole flow, and patient texts go out at the end.



● LIVE demonstrated in software

● PARTLY shown in part or not shown live

● PAUSED / NOT BUILT stated as off or missing

0:56:00 – 1:03:29 · 1:25:00 – 1:29:36

Your rules, inferred from your own people

WHAT IT DOES

- **Assignment logic from a conjoint survey:** your staff choose between clinicians in sample scenarios, and Axle infers the logic you actually use. You then review it and tune it during the pilot.
- They recommend **fewer than a dozen factors**, ideally six or seven, set up as a decision tree rather than weighted scores. That keeps every recommendation explainable.
- **Services** map one-to-one to HCHB service codes, each with a duration, priority, disciplines and licences.
- **Territories:** a soft default per team or per clinician. A clinician can still be scheduled outside it.
- **Shifts and blocks** with addresses and recurrence. The route optimizer plans around a mid-day appointment at its actual location. An “out sick” shift starts a call-out.

FOR COMPASSUS

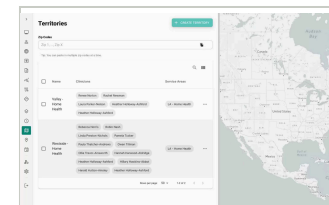
- **Workday integration isn't built.** They reviewed the APIs and want to pull availability from there instead of asking clinicians to enter it twice.
1:26:33
- Evan wants mandatory weekly availability from every clinician. Axle advised making it optional at first: it matters only where someone other than the clinician sets times. Evan's point stands, and this is a design decision for us to make.
- Productivity points by service code and targets by licence can be imported or kept in Axle.

“The algorithm needs to be explainable. You cannot have a true black box.”

Adam Stansell, Axle · 1:01:21

1:28:40 Shift blocks for a clinician or a whole team, with an address and recurrence

1:25:37 Services mapped to HCHB codes: duration, priority, disciplines, licences



1:25:55 Territories by zip, with the clinicians who serve them

1:26:46 Clinician schedules: working shifts by day

1:27:46 Shift settings: covered territories, start and end location, virtual or in-clinic

The integration they built, and why it's off

WHAT THEY SAID

- Read from the HCHB log ship: patients, service codes, clinicians, visits, frequencies and orders, auth and tasks. Write visits and tasks back by RPA, or by computer-use agents as those mature.
- **The bar:** a new SOC must reach Axle within about 10 minutes. HCHB's fastest refresh is 15 minutes, and when a refresh runs slow HCHB simply skips that cycle. Their Epic customers get a reliable 15 minutes, which is tolerable because it is *predictable*.
- They won't go live on data they can't time. HCHB's move to Databricks is the long-term fix; Compassus warned that HCHB "still can't get off Citrix."
- Suggested splitting the refresh by object: orders and frequencies fast, heavy documentation slow.

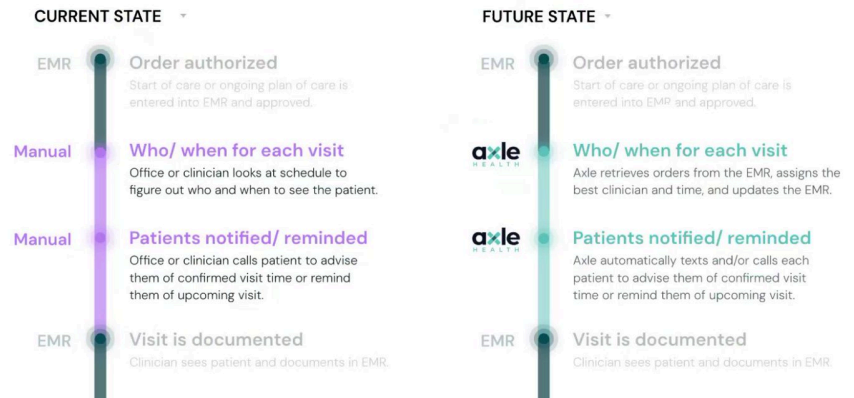
FOR COMPASSUS

- **New patients:** our intake platform (built with Commure, HL7 and APIs possible) can feed demographics and ordered disciplines in real time. That was enough for Axle to create the patient and episode and generate visits.
- **Routine visits still depend on HCHB.** Frequencies are written in the field and plotted by a scheduler workflow in HCHB, so at 3 hours Axle can't recommend days for new orders promptly.
- Compassus has cut refresh from 6 hours to 3 once already, and the data architecture work is the lever. Axle offered to think it through with us.
- RPA is the only write path into HCHB. Compassus is comfortable with it across several initiatives.

"We won't actually go live with somebody if we can't give any data guarantee... we need to be able to tell them when they will be in Axle."

Adam Stansell, Axle · 0:20:01

Axle's bi-directional integration engine automates manual work without disrupting EMR workflows



0:45:43 Their model: Axle takes over after the order is authorized, sets who and when, notifies patients, and the EMR keeps documentation

NEW PATIENT	Intake platform Commure build	API / HL7 real time →	Axle patient, episode, visits
ROUTINE ORDERS	HCHB frequency written	log ship 3 h today →	Axle needs ≤ ~15 min
WRITE BACK	Axle assignment, date, time	RPA →	HCHB visits, tasks

10 min

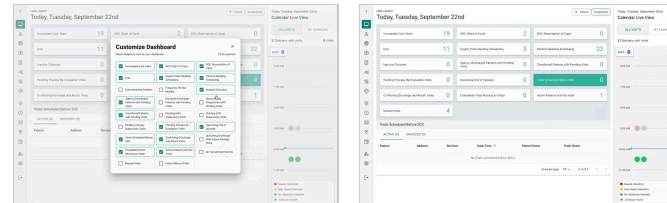
Axle's threshold for new-patient data. Compassus runs HCHB at 3 hours. The intake feed covers new patients; routine orders need a faster HCHB refresh, or order data taken from somewhere else, before day-of-week recommendations can work.

0:19:11

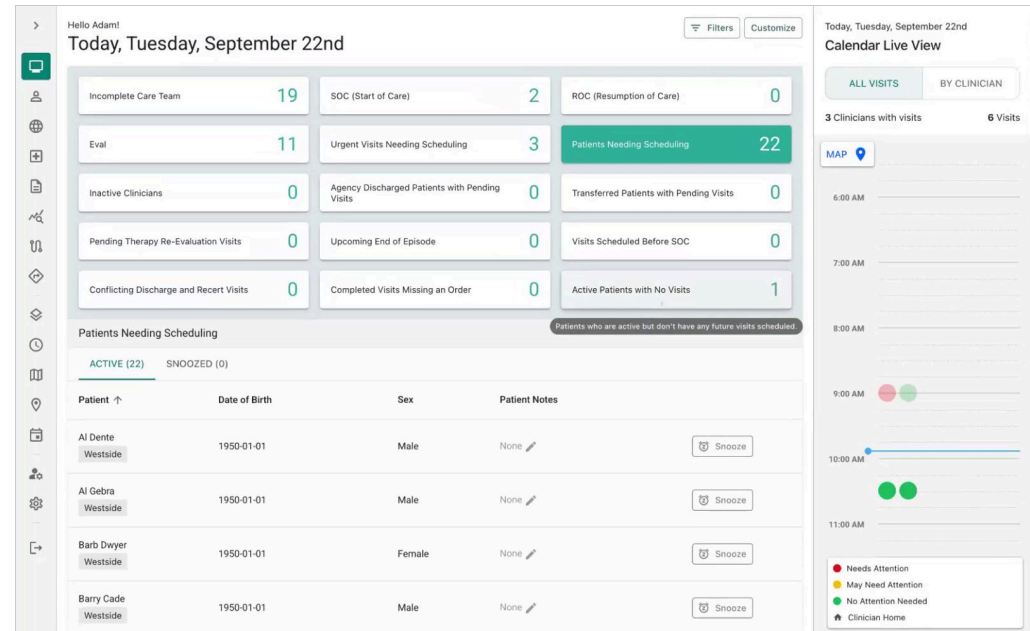
WHAT IT DOES

- FOR COMPASSUS**

- 0:50:04 Home dashboard: 15 work queues and a calendar live view



0:55:01 “Visits scheduled before SOC” and missed-visit queues



A care team per discipline, with every factor shown

WHAT IT DOES

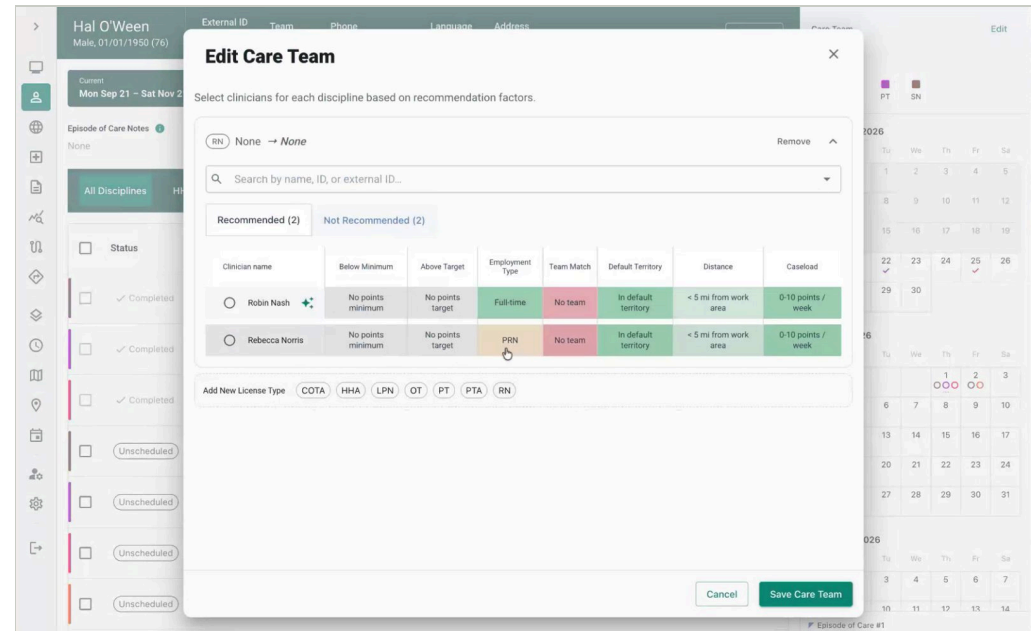
- For a new patient, Axle recommends the care team, one per discipline (RN, PT, OT, HHA and so on), with the factors laid out as a colour-coded table: care-team match, team, territory, top of licence, capacity, employment type, distance, caseload.
- It accounts for PTO and how full each clinician already is. It prefers top of licence (an OT over an OTA) and the less expensive licence where equivalent.
- Care team first; then every visit inherits it unless someone overrides.

FOR COMPASSUS

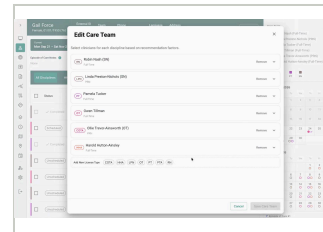
- Clinician-facing rationale is deliberately limited.** Clinicians see the reasoning for decisions they own (day, time). Assignment reasoning goes to the scheduler or manager, to avoid “why me and not Evan?” disputes. That runs against Colin’s wish for clinicians to see why.

1:07:41

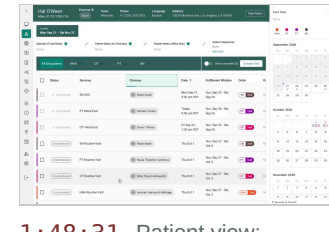
- Axle sees the scheduler as “a salesperson for capacity,” with the data needed to persuade a clinician to take a patient.



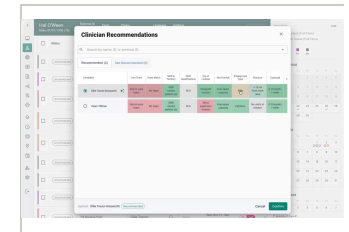
1:48:43 Care-team recommendations: each factor as a pass/fail cell, so the choice explains itself



1:48:19 Care team by discipline, with licence types to add



1:48:31 Patient view: episode, visits, fulfillment windows, orders and the episode calendar



1:49:19 Per-visit clinician recommendation for an OT visit

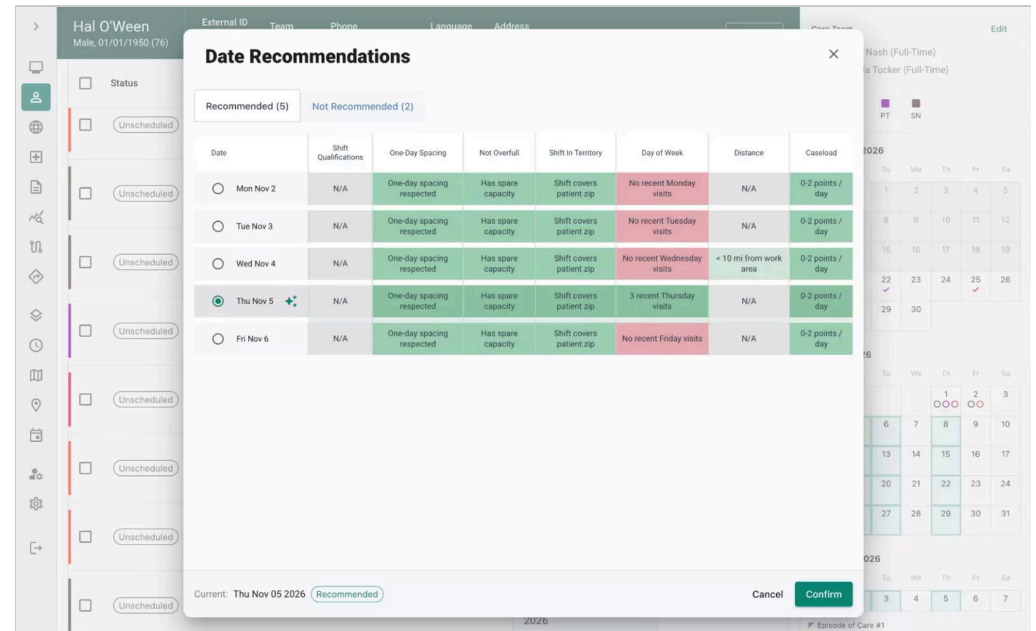
Bulk out the episode inside each visit's window

WHAT IT DOES

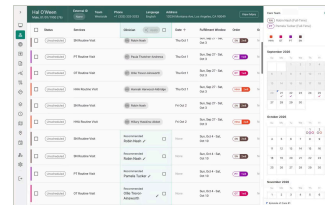
- Every visit carries a **fulfillment window**, such as the Medicare week Sunday to Saturday. Date recommendations pick the best day within the window while optimizing the whole episode.
- The rules shown were spacing between visits of the same order (no two SN visits on one day for a 2w8), front-loading early in the week, keeping a patient's usual weekday, and grouping patients geographically.
- Episode-level gaps such as a recert in the last five days are flagged even when no visit exists yet (from their return).

FOR COMPASSUS

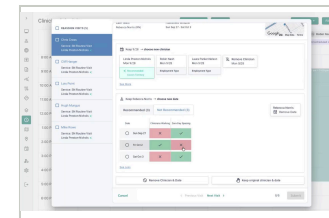
- This is the step our HCHB scheduler workflow does by hand today, and the one the 3-hour data lag hurts most.
- LUPA thresholds weren't shown on screen. Evan named LUPA and PDGM management as value levers; ask to see LUPA pacing on the shared scenario.



1:50:28 Date recommendations for a visit: spacing, capacity, territory, weekday history, distance, caseload



1:49:01 Bulk-out visits with recommended clinicians, ready to apply



1:45:43 Friday rejected: a same-order visit already falls that day

Optimize, finalize, and the patient is told

WHAT IT DOES

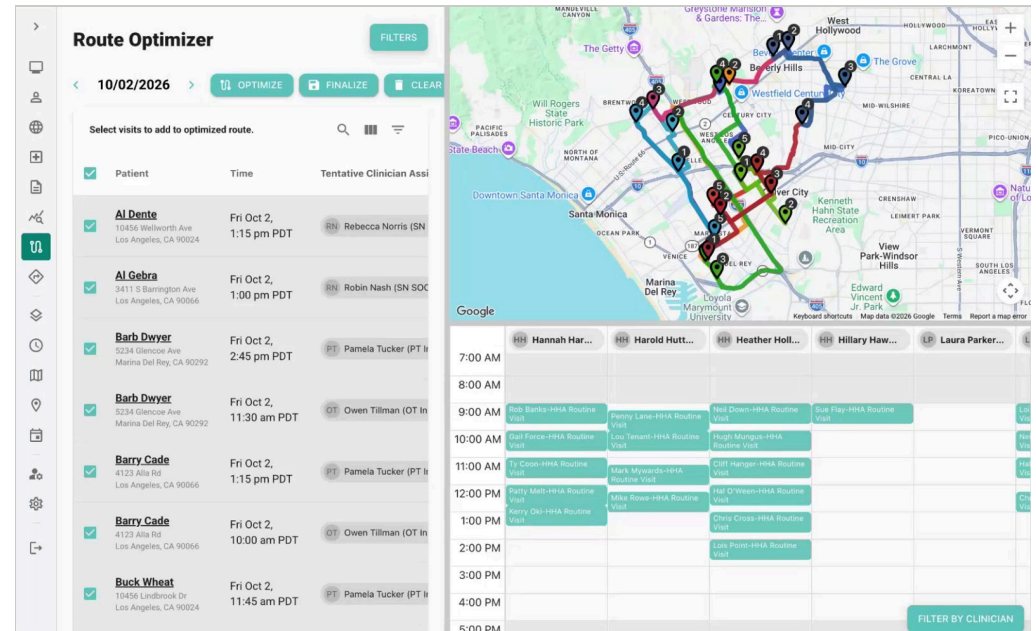
- The route optimizer sequences a day, or many clinicians' days, around shift blocks, patient constraints and facility hours. Usually the clinician runs it the night before: *optimize*, then *finalize*.
- Finalize sends each patient an SMS with an arrival **window** rather than a minute. The clinician works in sequence, not clock times.
- Patients have no app. Texts run through Twilio under a BAA, and replies route to the clinician's authenticated mobile app. Voice and email are also available.
- Patient availability starts fully open. Anyone can add constraints, standing ("dialysis Fridays") or for a single visit.

FOR COMPASSUS

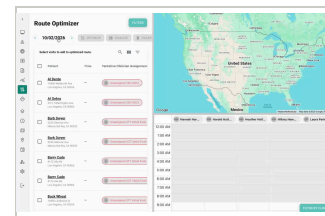
- **Their stance is confirm, don't ask.** Asking patients "can I come at 9?" loses the 9am slot, so negotiation moves to the prior visit and replies go to the clinician. It cuts the 30–60 minute evening call round without handing scheduling to a bot. 1:54:34
- Evan's north star is an agentic AI that captures patient availability and reschedules. Axle has self-scheduling in production but advises against it; if we use it, collect when patients are unavailable, not when they are free.
- Mobile app not shown live; videos and training guides promised.

"It shouldn't be a phone call... but it also shouldn't be an automated text bot where they can't push back."

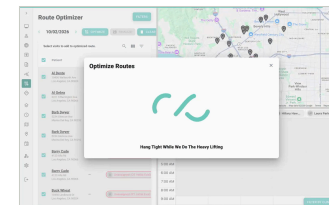
Adam Stansell, Axle · 1:57:15



1:56:25 Route optimizer: routes by clinician on the map and a timeline of the day



1:50:58 Selecting visits to optimize for a day



1:51:43 Optimized routes; finalizing texts every patient

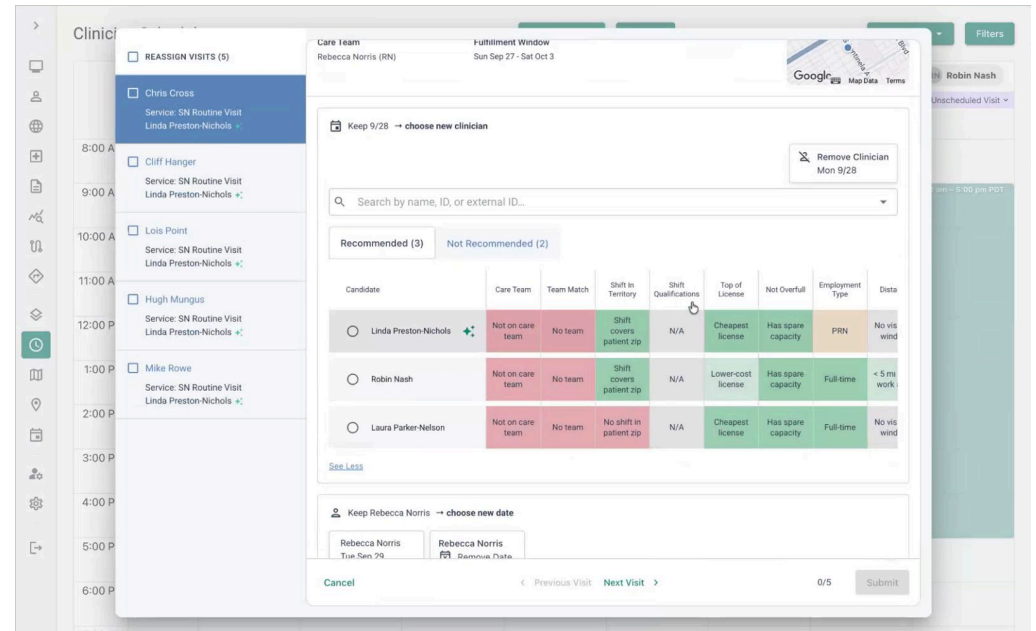
A call-out, handled in seconds

WHAT IT DOES

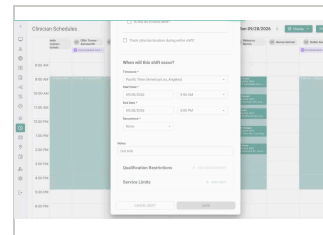
- Mark a shift “out sick” and the clinician’s visits open in a reassign flow, one visit at a time. Choose: keep the date and change the clinician, or keep the clinician and change the date. Their return adds a third option: switch the service code (an LPN visit to RN).
- Every option comes with its reasoning shown: territory, licence cost, capacity, employment type, distance, caseload, spacing.
- Accept all, submit, and the visits move. The covering clinician gets a push notification by default; for a PRN, the scheduler can call first.
- If nobody fits, Axle still names the best option and prompts the office to call (from their return).

FOR COMPASSUS

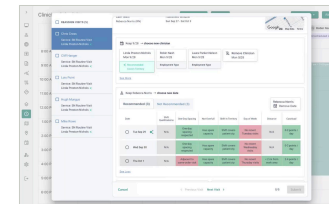
- This is the only live run of common question 6 in the field so far.
- They front-load within the week, so a missed Monday can land later in the week. Colin’s concern about brittleness was answered with front-loading and sequence over exact times.



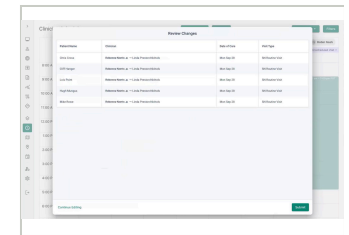
1:44:40 Reassign five visits: candidates scored on territory, licence cost, capacity, employment type and distance



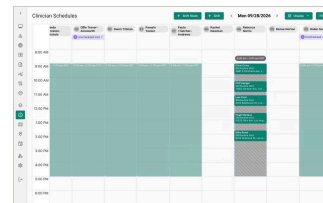
1:43:55 The trigger: a shift marked “Out sick”



1:46:01 Or keep the clinician: Tuesday and Wednesday preferred for spacing



1:46:25 Review all changes, then submit



1:47:13 The day before reassignment, shown on the schedule

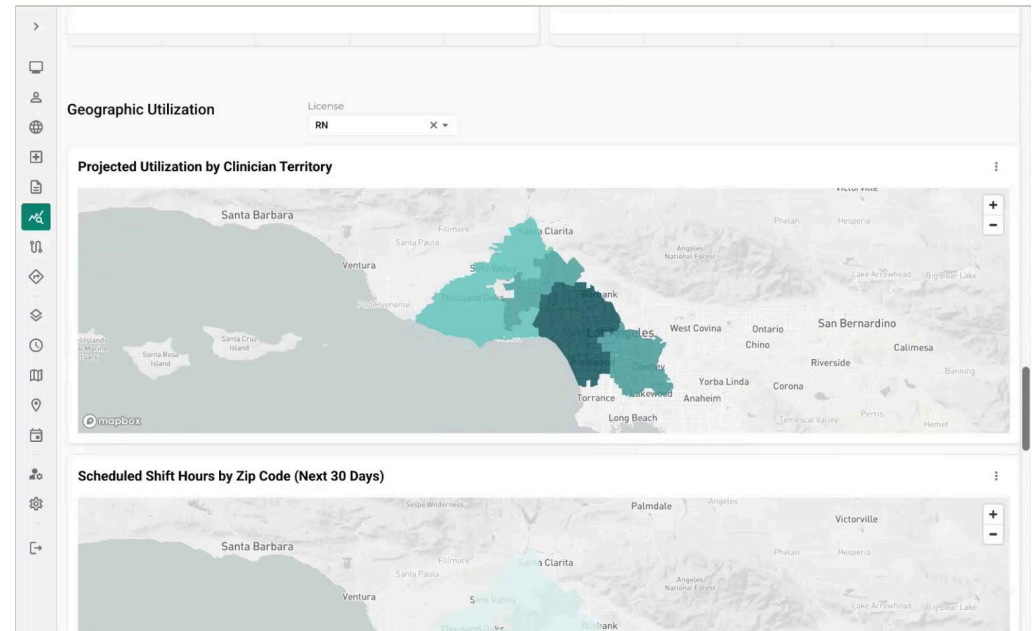
Capacity by zip, and proof the logic works

WHAT IT DOES

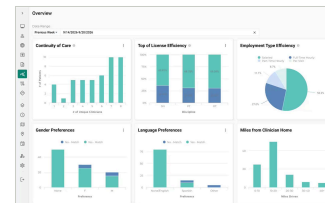
- **Projected utilization by clinician territory and zip:** historical demand, trends and seasonality set against actual capacity (PTO, reduced hours). It serves three audiences: recruiting (hire in the Valley), clinical ops (move a clinician over), marketing (don't chase referrals where you're full).
- **Checks on the assignment logic:** continuity of care, top-of-licence efficiency, employment-type mix, gender and language match, miles from home.
- **Clinician stats:** mileage, on-time rate, completion, call-out rate, patient ratings, and points utilization for the last and next four weeks against target.

FOR COMPASSUS

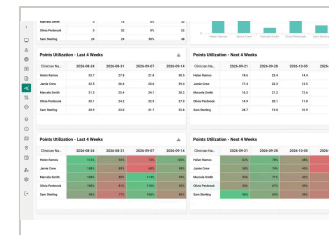
- Michael described the intake platform as the place where marketers, intake, insurance and ops come together. What's missing is capacity insight at accept or decline. Axle wants to feed and take in our marketing forecasts both ways. "A blank canvas conversation."
- 1:25:01
- Common question 1 (the referral decision) came closest to an answer here, though a 10am referral wasn't walked through live.



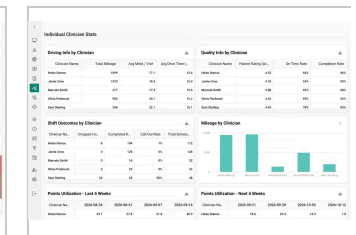
1:21:46 Projected utilization by clinician territory, plus scheduled shift hours by zip for the next 30 days



1:06:43 Checks on the logic: continuity, top of licence, employment type, preferences, miles



0:59:10 Points utilization, last four weeks and next four



0:58:28 Clinician stats: mileage, quality, outcomes

Impact and positioning

CLAIM	AS STATED	OUR READ
Clinician productivity	+17% Baseline is the 3 months before rollout against 3 consecutive months after full rollout	The only vendor with a stated baseline method. Still needs a named customer and census.
Scheduler time per visit	-74% "Schedulers about four times more efficient," fully realized only under a central model	Matches our plan to centralize schedulers. Branch-level savings will be smaller.
Mileage cost	-32% Target for Compassus: 5–10 million of our 45 million miles	\$3–6M a year at \$0.60/mile if it lands. Most of it comes from assignment and day grouping, not just routing.
Total opportunity	"At least \$100M in expense that could be saved" at our size, extrapolated from \$50M/yr per 1,000 clinicians	Directional only. Build our own case from the pilot.
Focus	Only scheduling, communications and workforce management. No scribe, billing or EMR	Credible, and it lowers the risk of their attention drifting.
Credentials	Deck shows SOC 2 Type 2, HIPAA, Y Combinator and other named investors	Size, funding stage and runway were not given on the call.

Axle drives meaningful impact



+17%

Increase in clinician productivity



-74%

Decrease in scheduler time per visit



-32%

Decrease in mileage costs



0:47:28 Their impact slide: +17%, -74%, -32%

Axle streamlines operations across 3 key areas

Workforce optimization

Forecast staffing needs to balance caseloads and increase referral acceptances



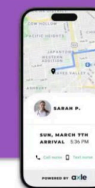
AI Scheduling

Optimize case assignment and scheduling to improve productivity



Communications

Reduce patient no-shows and missed visits with next gen patient engagement



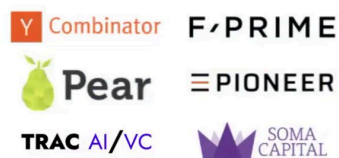
0:42:04 Three product areas: workforce optimization, AI scheduling, communications

The right team

Deep expertise across healthcare and technology



Backed by leading investors



Certifications & Awards



0:49:19 Team, investors and certifications

Question by question

• ANSWERED 6 • PARTLY 8 • NOT ANSWERED 0 • NOT ASKED 4

QUESTION	STATUS	WHAT WE HEARD	WHEN
Common 01 The referral decision	• PARTLY	Zip-level projected utilization and the recommended clinicians for a new patient; built to guide recruiting, ops and marketing. A live 10am referral wasn't walked through.	1:18:10
Common 02 The episode, not the visit	• ANSWERED	Fulfillment windows, the Medicare week, front-loading, same-order spacing, and queues for end of episode, therapy re-eval, and recert against discharge. LUPA wasn't shown on screen.	1:45:02
Common 03 HCHB freshness, with a number	• ANSWERED	15 min at best and unreliable; up to 30 min in practice. Needs ~10 min for new patients. 3 hours is too long for routine plotting.	0:19:11
Common 04 Advise versus decide	• ANSWERED	Designed to decide, then rebuilt to advise. Every customer keeps a human in the loop, and they argue for keeping one. No customer named who moved the line.	1:14:22
Common 05 The clinician says no	• PARTLY	Accept/decline or reassignment requests, set by role. Disputes go through the manager, with the data to hand. No 60-day model learning described.	1:07:41
Common 06 A Tuesday that goes wrong	• ANSWERED	Run live: out-sick shift, five visits reassigned with reasoning, push notification. If nobody fits, Axle still names the best option and prompts a call.	1:43:00
Common 07 What you don't do	• PARTLY	HCHB integration off; Workday integration not built; dynamic incentives in development (return). Not asked as three items with dates.	1:26:33
Common 08 What breaks at our size	• PARTLY	HCHB replication doesn't scale horizontally, so larger databases refresh slower. Largest deployment 11k ADC (return). We'd be about twice that.	0:30:00
Common 09 The one number you'd stake it on	• PARTLY	+17% / -74% / -32% with a stated 3-month baseline. No named customer on the call.	0:46:26
Common 10 Where you'll look worse	• NOT ASKED	Integration was the obvious answer and was discussed openly.	—

QUESTION	STATUS	WHAT WE HEARD	WHEN
Axle 01 What lags, your threshold, what must change	● ANSWERED	New-patient data within ~10 min, and predictably. HCHB skips a refresh when it runs slow. Databricks is HCHB's intent, not a commitment to Axle.	0:19:59
Axle 02 Turn it back on for Compassus, with a date	● PARTLY	Yes for new patients via our intake feed. Routine orders depend on our HCHB cadence. No date given.	0:22:15
Axle 03 "Always bi-directional" versus switched off	● NOT ASKED	The contradiction in the return was not raised.	—
Axle 04 Which EHRs; the 11k ADC deployment	● PARTLY	Epic customers get reliable 15-minute data. The rest of the EHR mix wasn't covered.	0:31:25
Axle 05 The real partnership proposal	● NOT ASKED	Return: referral credits, co-built features permanently discounted, open to strategic terms.	—
Axle 06 Incentives: date and interim	● PARTLY	Compassus raised incentives for taking out-of-zip patients. Axle's dynamic pricing (per the return) is in development with no date.	0:38:33
Axle 07 Who is on the call; who runs our deployment	● ANSWERED	CTO, head of product, head of professional services and sales on the call; Adam led the demo.	0:00:03
Axle 08 Company size, funding, home health focus	● NOT ASKED	Deck lists investors and certifications; 80% home health (return). Size and funding not given.	0:49:19

What to weigh

HIGH

Go-live depends on our data

Axle won't launch on a feed it can't time. The intake feed covers new patients; routine orders need faster HCHB data. The pilot date is really a data-architecture date.

HIGH

Write-back into HCHB is unproven here

RPA or computer-use write-back exists but has no live HCHB customer. Detection, repair SLA and conflict handling for a customer our size are all untested.

MEDIUM

Patient engagement philosophy

Confirm-don't-ask is proven but conservative. Our agentic two-way vision would be layered on top of it, and Axle will push back. Decide which we want before the on-site.

MEDIUM

Clinician visibility of the “why”

Assignment reasoning goes to schedulers and managers by design. Colin wants clinicians to see it. It can be configured, but it goes against their advice.

MEDIUM

Scale

At 24k ADC we'd be about twice their largest deployment. Performance of the logic and data volume at our size need proof on the pilot.

MEDIUM

Commercials unasked

Partnership, size and funding weren't covered. The return's “referral credits and discounted features” is thinner than the design-partner investment we're offering.

MEDIUM

Workday and availability

No Workday integration yet. Evan wants mandatory availability; Axle wants optional adoption. That must be settled in the pilot design.

MEDIUM

Location tracking policy

By default Axle tracks only trips to a patient, when the clinician starts one. Whole-shift tracking is available, and choosing it is a privacy and HR decision for us.

IN THEIR FAVOUR

Home health fluency and candor

80% home health, our vocabulary unprompted, a live recovery flow, change management learned the hard way (“rip the band-aid off” was a mistake), and they switched their own integration off rather than ship a poor experience.

What happens now

Agreed on the call

1. **AXLE**

Send mobile app videos, training guides and screenshots of the clinician experience.

2. **COMPASSUS**

Confirm exact home health clinician count, scheduler split by service line and missed-visit rate.

3. **BOTH**

A dedicated “blank canvas” session on capacity forecasting and a two-way link with the intake platform.

4. **COMPASSUS**

Look at HCHB refresh cadence by object (orders and frequencies faster) inside the data architecture work. Axle offered to help think it through.

5. **COMPASSUS**

Decide the on-site shortlist for the week of Oct 19.

Recommended for the on-site

- A joint data plan: intake feed scope, target HCHB cadence for orders and frequencies, and a date to turn write-back on.
- Run the shared scenario end to end, including LUPA pacing and a 10am referral decision against capacity.
- The RPA write-back story: detection when HCHB changes a screen, repair SLA, conflict handling.
- Partnership terms that match our design-partner investment; company size, funding and runway.
- A named reference at 5k+ ADC with the before-and-after on productivity and mileage.
- Patient engagement: agree the boundary between Axle’s confirm model and our agentic two-way ambition.
- Clinician-facing assignment rationale, and a location-tracking policy for the pilot.
- A Workday integration plan and whether the availability model is mandatory or optional.

Who was there

Compassus

Evan Kramer

Leads Innovation & Operational Excellence

Colin Highland

Initiative lead · PT, ~25 years in home health

Michael Thompson

VP, Operational Excellence

Paige Huffman

Senior Project Manager, Operational Excellence

Axle Health

Adam Stansell

Led the demo and discovery

Romit

Head of Product · joined late, blocked by Teams

David

Sales

Jessica

CTO

Eshon

Head of Professional Services

Built from the Teams recording (2:01:00) with a machine transcript and extracted screens, plus Axle’s questionnaire return where noted. Speaker attribution is inferred from context where the transcript has no labels.

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